

Patient Name:

Date:

Physical Therapy Script: ACL Reconstruction Protocol

1-2 WEEKS

Goals: *Hyper-extension equal to opposite side and flexion progressing to = or > 100°, progress patient off crutches with a normal gait pattern, decrease swelling, inflammation and pain.*

Gait training, ice or **cold therapy** device frequently; convert to functional brace when quads are functionally working

Progress PROM to patient's tolerance (continue with patellar mobilizations prone extension, flexion stretches)

Continue with quad sets (perform in standing), multi-angle isometrics, EMS or biofeedback to quadriceps

SLR in side lying for hip abduction/adduction. Continue in supine with knee extended

Active hamstring curls 0° to 90° in prone and continue AROM 90° to 30° for extension

Initiate stationary bike with no resistance for ROM

No isokinetic dynamometer (Cybex, Kin Com, Biodex) for strengthening

3-4 WEEKS

Goals: *Achieve full hyperextension, achieve flexion to = or > 120 degrees*

WBAT- gaits training as appropriate to normalize gaits, and continue with stationary bike

Continue three-way SLR (weights as tolerated), or Cybex hip abduction/adduction only

Toe-raises in sitting/standing to strengthen soleus and gastrocnemius

Wall sits as up to 70° knee flexion/mini squats

BAPS board for balance

Lateral step-ups 1-4 " block

Be aware of patella tendonitis- if present defer TKE and squats

Once the incision site is completely healed, instruct in pool exercises for ambulation/ROM- if a pool is available

Jogging in pool with wet vest (if available)

Continue ice or cold therapy device after exercises and at the end of the day prn

5-6 WEEKS

Goals: *Progress to hyperextension and flexion to = or > 130 degrees*

Full weight-bearing – normalized gait

Lateral step-ups 2 – 5" block, can use Sport Cord

Continue with stationary bike, Cybex multi-hip toe-raises in sitting/standing with weight as tolerated

Continue SLR and hamstring curls with progressive weights, BAPS board

Add single-leg stance on mini trampoline and retrowalking

May use electric stim or biofeedback if indicated

(Continue jogging in pool or free-style swimming – if available)

7-12 WEEKS

Goals: *Patient's ROM to normal and very minimal to no effusion.*

(If patient is progressing, may decrease treatment frequency per week i.e. 1x/week)

Continue lateral step-ups (4 – 8 “ block), toe-raises, bike (emphasizing interval training), and pool exercises

Add lunges if no patellofemoral symptoms

Initiate open-chain knee extension 90° to 30° , with progressive weights @ 8 weeks if no PF symptoms

Isokinetics sub maximally, hamstring full range and quadriceps 90° to 30°

Fitter exercise for proprioception

Nordic track

13-20 WEEKS

Goals: *Begin jogging at three months*

(If patient is progressing may decrease PT frequency per month i.e. 1x/month through 6 months post-op)

Begin progressive resistance exercises for knee extension and flexion (full range)

Continue hip abduction/adduction, toe-raises, soleus and gastroc strengthening, hip extensions and leg press

Continue Fitter, bike (utilizing interval training), lateral step-ups

Isokinetics for quadriceps and hamstrings, full range, progress to maximal

Begin plyometrics, lateral shuffles and carioca

Initiate jogging if full ROM and no effusion

21-28 WEEKS

Goals: *Begin sporting activities at 4-6 months, discuss with MD first.*

Advance plyometrics; i.e. double and single leg hops, etc.

Initiate sport-specific skills progressing from straight line running to cutting, pivoting, twisting, if required

Notes:

- 1) Any signs/symptoms of laxity contact the MD
- 2) Any signs of infection or deep vein thrombosis contact the MD
- 3) *Kin Com Testing Protocol: if ordered, isokinetic testing is to be done between the range of 90° - 125° degrees of knee motion
Testing eccentrically at 60° / second, and concentrically at 90° / second.
Compare the involved knee versus the non-involved knee to determine the % difference

Signature _____

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